

How to Open an In-Home Care Agency in Arkansas

A step-by-step guide to the Arkansas Private Care Agency license — what changed under Act 853 of 2025, how to get licensed by the Department of Health, screening caregivers, finding clients, and getting paid

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The Arkansas difference — read this first

Arkansas licenses non-medical personal care agencies as Private Care Agencies. The license comes from the Arkansas Department of Health (ADH), Health Facility Services, under the Rules for Private Care Agencies (Ark. Code Ann. § 20-10-2301 et seq.). It is separate from the Class A, B, and C home health agency licenses, which cover skilled nursing and therapy. One recent change matters a great deal: Act 853 of 2025 moved private care agency licensing to ADH, eliminated the old Department of Human Services (DHS/DPSQA) certification step, made Medicaid participation a separate provider enrollment, and removed the old requirement to keep multiple regional offices — so an agency now maintains a single primary office in Arkansas. This guide walks you through the ADH license, the people you need (an administrator, a registered nurse, and aides who complete Arkansas's forty-hour training), Arkansas's strict background and maltreatment-registry checks, how you find clients, and how you get paid — privately and through Arkansas Medicaid, ARChoices, PASSE, and IndependentChoices.

How to Use This Guide

This guide takes you from idea to an operating Arkansas in-home care agency. Read it once start to finish, then use it as a checklist. Everything here is general information for prospective Arkansas agency owners — not legal, tax, or licensing advice. Requirements, fees, and phone numbers change, and Act 853 of 2025 recently reshaped the rules, so verify each step with the agency named before you rely on it. Where this guide gives a figure, treat it as a starting point to confirm, not a final answer.

The one-paragraph version

Register your business with the Arkansas Secretary of State, get an EIN, and insure it. Name a qualified administrator and secure a registered nurse. Apply to the Arkansas Department of Health, Health Facility Services, for a Private Care Agency license on the prescribed forms, and pass the on-site ADH survey. Build your team: every owner and caregiver clears state and national criminal background checks and the adult and child maltreatment registry checks, and every personal care aide completes at least forty hours of training — twenty-four classroom hours and sixteen hours of supervised practical training under a registered nurse — before working independently. To serve Medicaid clients, enroll separately as an Arkansas Medicaid provider, enroll each caregiver for a PIN, and set up Electronic Visit Verification. Then deliver personal care under a written service plan, privately or through Medicaid personal care, ARChoices, PASSE, or IndependentChoices.

The Path at a Glance

Ten steps take you from idea to an operating Arkansas in-home care agency:

- 1. Form the business.** Register with the Arkansas Secretary of State; get an EIN; open a business bank account.

2. **Name an administrator.** A physician, a registered nurse, or someone with at least one year of supervisory or administrative experience in home health or a related program.
3. **Secure a registered nurse.** The RN supervises the practical part of aide training and takes part in each client's service plan.
4. **Put protection in place.** Professional and general liability insurance and workers' compensation.
5. **Write your policies.** Adopt the policy and procedure manual, client forms, and caregiver handbook built to the Arkansas Rules for Private Care Agencies.
6. **Apply to ADH.** Submit your Private Care Agency license application on the prescribed forms to ADH, Health Facility Services.
7. **Pass the ADH survey.** An on-site survey to the Rules for Private Care Agencies; then ADH issues your license.
8. **Screen and train.** Criminal and maltreatment-registry checks on everyone; the forty-hour aide training before independent work.
9. **Enroll with Medicaid (to bill it).** Enroll as an Arkansas Medicaid provider, get each caregiver a PIN, and set up EVV.
10. **Serve clients and grow.** Deliver personal care under a written service plan — private pay, Personal Care, ARChoices, PASSE, or IndependentChoices — and build referrals.

Part 1 — What This Business Is

An in-home care agency sends caregivers into clients' homes to help with everyday life: bathing, dressing, grooming, toileting, transferring, and eating (personal care); light housekeeping, laundry, meals, and errands (homemaker and supportive services); companionship; and accompanying clients to medical appointments. In Arkansas, an agency that provides hands-on personal care is licensed as a Private Care Agency — the credential this guide is built around.

Arkansas is a solid market for in-home care: a large rural and small-metro population that wants to age at home, and a Medicaid system — Personal Care, ARChoices, PASSE, and IndependentChoices — that pays for a great deal of it. The trade-off is a real regulatory structure: an ADH license and survey, a forty-hour aide-training requirement, and some of the stricter background and maltreatment-registry checks in the region. Meeting those standards is exactly what makes hospitals, care coordinators, and families trust you.

A useful way to think about it: the private-pay side of the business is a straightforward service company, while the Medicaid side is a clinical operation organized around a written service plan and, for ARChoices, the state's own registered nurse. Many successful Arkansas owners run both at once — private-pay clients keep cash flowing while the Medicaid programs reach the larger population. Deciding which you are building first tells you what to prioritize: speed and marketing for private pay, or your license, your policies, and your aide training for the Medicaid path.

The service lines, in plain terms

- **Personal care:** hands-on, medically necessary help with the activities of daily living — the core of a licensed private care agency, billable to Arkansas Medicaid.
- **Homemaker and supportive services:** housekeeping, laundry, meals, and companionship — often private pay.
- **Instrumental support and errands:** shopping, meal prep, and accompanying clients to medical appointments, as the service plan directs.
- **What you don't do:** skilled nursing and therapy — those need a separate Class A, B, or C home health agency license.

Part 2 — The Arkansas License: Private Care Agency

The credential you need to run a personal care agency in Arkansas is a Private Care Agency license from the Arkansas Department of Health (ADH), Health Facility Services, under the Rules for Private Care Agencies (Ark. Code Ann. § 20-10-2301 et seq.). This is different from the Class A, B, and C home health agency licenses, which cover skilled nursing and therapy — those are a separate credential you do not need to provide personal care.

Getting this distinction right early saves months. If your honest plan is hands-on help with bathing, dressing, meals, and the other activities of daily living, you are building a private care agency and you apply to ADH for that license. If you intend to provide skilled nursing or therapy, that is a different business with a different license. Trying to straddle both without the right credentials is the fastest way to a compliance problem, so decide clearly and license accordingly.

What Act 853 of 2025 changed

If you have read older guidance, some of it is now out of date. Act 853 of 2025 (HB1439) reshaped how private care agencies are regulated. Licensing moved to the Arkansas Department of Health, and the old Department of Human Services (DHS/DPSQA) certification step was eliminated. Participating in Medicaid is now a separate Arkansas Medicaid provider enrollment rather than a DHS certification. And the old requirement to maintain multiple regional offices was removed — an agency now only needs a single primary office physically located in Arkansas. Build your plan around the current rules, and confirm the details with ADH before you file.

A note on the Permit of Approval

A Permit of Approval (POA) is clearly part of the process for a Class B home health license. For a private care agency operating strictly under the personal care framework, whether a POA applies to your specific model is a question to confirm directly with ADH — do not assume every non-medical startup needs one, and do not assume it doesn't. Ask ADH about your exact plans.

Part 3 — Getting Licensed by ADH

You apply for your Private Care Agency license through the Arkansas Department of Health, Health Facility Services. The application centers on showing ADH that your agency is organized, staffed, and documented to deliver personal care safely.

Treat the survey as the moment your paperwork becomes real. Before the surveyor arrives, walk your own files as if you were them: is there a written service plan for each client, a completed criminal and maltreatment-registry check for each caregiver, and a forty-hour training record for each aide? Fixing gaps before the survey is far easier than answering for them after, and a clean first survey sets the tone for your relationship with ADH going forward.

1. **Apply on the prescribed forms.** Submit ADH's Private Care Agency application with your policies and procedures, proof of insurance, administrator information, background-check attestations, and your emergency preparedness plan.
2. **Pass an on-site ADH survey.** A surveyor reviews your agency against the Rules for Private Care Agencies — administration, the qualified administrator, aide training, the written service plan, documentation, and client care.
3. **Receive your license.** ADH issues the Private Care Agency license, on a calendar-year basis expiring December 31; you renew annually.

Build to the rule, and keep it current

Arkansas surveyors expect policies that reference the Rules for Private Care Agencies specifically — generic or other-state policies won't pass. Build your policy manual and your caregiver and client files to the Arkansas rules from the start, keep your license and (if you bill Medicaid) your Medicaid enrollment current, keep every background check, registry check, and the forty-hour training current, and stay ready for an ADH survey or complaint investigation at any time.

Part 4 — Your Team: Administrator, RN & the 40-Hour Aide Training

An Arkansas private care agency runs on three roles. Plan for all three from day one.

- **A qualified administrator.** The administrator must be a physician, a registered nurse, or a person with at least one year of supervisory or administrative experience in home health care or a related health provider program, and is responsible for the agency's day-to-day operation and compliance.
- **A registered nurse.** An RN supervises the practical, hands-on part of aide training and takes part in developing and updating each client's written service plan. For an ARChoices client, the Arkansas DHS registered nurse develops the person-centered service plan, and your agency coordinates with that DHS RN.
- **Personal care aides who complete the forty-hour training.** Before an aide works independently, they complete at least forty hours of training — at least twenty-four hours of classroom instruction and at least sixteen hours of supervised practical training provided by or under a registered nurse.

Develop your forty-hour aide-training curriculum early and have it ready for ADH — it is one of the concrete things a surveyor will look for, and you cannot put aides to work independently without it.

A practical tip: keep the three roles visibly staffed and documented. Name your administrator in writing and record how they meet the qualification; put your registered nurse's involvement in training and service planning on paper; and keep each aide's forty-hour training record, with the classroom and practical hours broken out, in their personnel file. Surveyors look for exactly these artifacts, and families and referral partners take comfort from knowing they exist.

Part 5 — Background Checks & Maltreatment Registries

Arkansas is strict about who provides direct care, and the checks apply to owners, principals, employees, and contract staff alike.

- **State and national criminal background checks.** Required under Ark. Code Ann. §§ 20-33-213 and 20-38-101 et seq. before a person has direct client contact, and repeated on the required schedule.
- **Adult Maltreatment Central Registry check.** Check the Adult and Long-Term Care Facility Resident Maltreatment Central Registry on every direct-care applicant.
- **Child Maltreatment Central Registry check.** Check the Child Maltreatment Central Registry as well — a routine part of Arkansas direct-care screening.
- **Act on what you find.** A disqualifying criminal result or a registry finding that bars direct care keeps a person out of a direct-care role; document each check and its determination and keep it confidential.

Screen before contact, and keep it on file

Complete every check before a new hire has regular contact with clients, not after. At a survey, ADH will expect to see, for each caregiver, that the criminal and maltreatment-registry checks were completed and cleared before client contact, that the person signed the required disclosure, and that the forty-hour training was done before independent work.

Part 6 — Setting Up to Operate

Form the business and name your leaders

Register your business with the Arkansas Secretary of State, get a federal EIN, and open a business bank account. Name your qualified administrator and secure your registered nurse. Under Act 853, you maintain a single primary office physically located in Arkansas, where your records are kept and available for ADH survey — you no longer need multiple regional offices.

Insurance, policies, and Medicaid enrollment

Carry professional and general liability insurance and workers' compensation coverage. Adopt a policy and procedure manual, client forms, and a caregiver handbook built to the Rules for Private Care Agencies. To bill Medicaid, enroll separately as an Arkansas Medicaid provider through the Provider Portal (the fiscal agent is Gainwell Technologies), enroll each caregiver for a PIN, and set up Electronic Visit Verification. Your written service plans, aide training, and RN involvement are your quality-control system.

Think of your administrator and your registered nurse as the two load-bearing roles. The administrator owns the business — the license, enrollment, insurance, records, billing, and hiring — and the RN owns the clinical side — aide training and the service plans. In a small startup one person may wear more than one hat, but both sets of responsibilities have to be covered, with enough authority and time to do the job. Underfunding either one is where new agencies most often get into trouble.

Part 7 — Finding Clients & Referrals

Two engines fill your schedule: families who pay privately, and the aging-and-disability network — including Arkansas's Medicaid programs — that steers people to agencies. Build both from the start.

- **Hospital discharge planners and case managers** send people home who need help — be the licensed agency they trust to prevent a readmission.
- **DHS county offices and ARChoices staff** connect eligible Arkansans to in-home services — key Medicaid referral partners.
- **The PASSEs** (the Provider-led Arkansas Shared Savings Entities) coordinate care for members with behavioral health needs or developmental disabilities.

- **Skilled nursing and rehab facilities** discharge clients who still need in-home support after therapy ends.
- **Hospices** refer both ways: they need aides for personal care, and you refer declining clients to them.
- **Assisted living communities, senior centers, elder-law attorneys, and Area Agencies on Aging** guide Arkansas families to trusted agencies.

Your PolicyReady Referral System for Arkansas lists named hospitals, skilled nursing facilities, hospices, senior communities, and elder-law attorneys across Little Rock, Northwest Arkansas, Fort Smith, Jonesboro, and Hot Springs, with the playbook for turning them into a steady stream of clients.

Referral relationships are earned, not announced. The people who send you clients — discharge planners, care coordinators, hospice liaisons — are putting their own reputation on the line, so they refer to agencies they trust to be reliable, responsive, and compliant. Show up on time, communicate clearly, handle problems without drama, and make the referrer look good to the family, and the referrals compound over time.

Part 8 — Getting Paid

In-home care in Arkansas is paid three main ways: privately by the client or family; by Arkansas Medicaid through Personal Care, the ARChoices waiver, the PASSE program, and IndependentChoices self-direction; and by other sources such as long-term-care insurance and Veterans benefits. Private-pay work can start fastest; the Medicaid programs reach a much larger client base.

Private pay

Set a clear hourly rate, put it in a written service agreement, and bill only for services actually provided. Private pay needs no Medicaid enrollment, and rates in Little Rock and Northwest Arkansas tend to run higher than in rural areas. Build in your real labor costs, including overtime and the applicable minimum wage.

Price private pay so it is sustainable, not just competitive. Your rate has to cover aide wages and overtime, payroll taxes, workers' compensation, training, insurance, scheduling and documentation software, and your own overhead — and still leave a margin. Underpricing to win early clients is a common mistake that makes an agency fragile the moment costs rise. A written service agreement that states the rate, the schedule, and the cancellation terms protects both you and the client.

Arkansas Medicaid — Personal Care, ARChoices, PASSE & IndependentChoices

To bill Medicaid you enroll as an Arkansas Medicaid provider and enroll each caregiver for a PIN. Medicaid pays for personal care under the State Plan (hands-on help with physical-dependency needs, with a monthly hour limit for adults absent an approved benefit extension); through the ARChoices in Homecare waiver, where the DHS registered nurse develops the person-centered service plan; through the PASSE program for members with behavioral health or developmental-disability needs; and through IndependentChoices, where the participant self-directs their care. You deliver only authorized services under the service plan and capture EVV on every Medicaid visit.

The practical takeaway is that Medicaid billing rewards discipline. Authorized services, a current service plan, an EVV-verified visit by a PIN-enrolled caregiver, and documentation that all line up are what get claims paid and keep you clear in an audit. Build that chain into your daily workflow from your first Medicaid client, and reconcile your EVV, your visit notes, and your billing regularly so small mismatches are caught early.

EVV is required — set it up before you bill Medicaid

Arkansas requires Electronic Visit Verification for Medicaid personal care, attendant care, and respite delivered in the home. Each caregiver must be enrolled with Arkansas Medicaid and have a PIN, and every visit is captured and verified electronically — claims are paid only when the visit is EVV-verified and the caregiver is enrolled. Set up EVV and train your aides on it before you take Medicaid clients, and reconcile your EVV records to your documentation and billing so claims are clean.

Part 9 — Your First 90 Days

1. **Before you apply:** Form the business, get your EIN and bank account, name your administrator, secure your RN, and bind insurance and workers' comp.
2. **Before you apply:** Adopt your policy manual, client forms, and caregiver handbook to the Rules for Private Care Agencies, and build your background-check, registry-check, and forty-hour training systems.
3. **Apply to ADH:** Submit your Private Care Agency application on the prescribed forms and prepare for the on-site survey.
4. **During review:** Recruit aides, complete their background and registry checks and the forty-hour training, and get ready for the survey.

5. **After licensure:** If billing Medicaid, enroll as an Arkansas Medicaid provider, get caregiver PINs, and set up EVV; take your first clients under written service plans; and start referral outreach to two or three hospitals, a hospice, and your DHS county office or a PASSE.

Part 10 — Common Mistakes to Avoid

- **Relying on out-of-date guidance.** Act 853 of 2025 moved licensing to ADH and eliminated the old DHS/DPSQA certification — build to the current rules.
- **Confusing the licenses.** A private care agency license is not a Class A, B, or C home health license; personal care and skilled care are different credentials.
- **Putting aides to work before the forty-hour training.** Twenty-four classroom hours plus sixteen practical hours under an RN come first.
- **Skipping or delaying the checks.** Complete the criminal and adult and child maltreatment-registry checks before any client contact.
- **Assuming the license lets you bill Medicaid.** Medicaid is a separate provider enrollment, and each caregiver needs a PIN.
- **Skipping EVV setup.** Every Medicaid in-home visit must be EVV-verified; set it up before you bill.
- **Providing skilled care beyond your license.** Skilled nursing and therapy require a separate home health agency license — otherwise refer it out.

Part 11 — Staying Compliant & Growing

Once you are open, compliance is ongoing: renew your license each year; keep every caregiver's criminal and maltreatment-registry checks, forty-hour training, and competency current; keep service plans updated and RN involvement documented; if you bill Medicaid, keep your enrollment, caregiver PINs, and EVV current; maintain insurance; and run a quality program that tracks complaints, incidents, and satisfaction and fixes what they show. Keep a compliance calendar so nothing lapses, and stay ready for an ADH survey at all times.

The agencies that stay out of trouble treat compliance as a weekly habit rather than an annual scramble. A short recurring review — which caregivers are due for a background-check or registry refresh, which training is coming due, which service plans need updating — catches small lapses before they become survey findings. Your administrator and your registered nurse should own that review together, because the administrative and clinical deadlines are intertwined.

Growth comes from three moves: deepen referral relationships — especially with hospitals, DHS county offices, and the PASSEs — until you are the agency they call first; add payers (layer Medicaid, ARChoices, PASSE, and IndependentChoices onto private pay, then long-term-care insurance and VA benefits); and expand your service area and team at a pace your administrator and RN involvement can actually support. As you grow, don't let volume outrun your background checks, your training, or your documentation — those are the things a survey looks at hardest, and the things that protect your clients.

Part 12 — What It Costs to Start

Plan for the following, and gather real quotes rather than guessing. Startup costs for an Arkansas in-home care agency typically run into the tens of thousands of dollars once you add licensing, insurance, staffing, training, and working capital — confirm current figures for your plan:

- **ADH license:** the Private Care Agency application and license fee, plus preparing for the on-site survey — confirm the current amount with ADH, Health Facility Services.
- **Registered-nurse involvement:** an employed or contracted RN to supervise the practical part of aide training and take part in service plans.
- **Aide training:** delivering the forty-hour curriculum (twenty-four classroom hours plus sixteen practical hours under an RN) to each aide.
- **Insurance:** professional and general liability and workers' compensation — get quotes for your size and services.
- **Business formation:** registration with the Arkansas Secretary of State, plus your accounting setup.
- **Policies and documents:** your policy and procedure manual, client forms, and caregiver handbook built to the Rules for Private Care Agencies.
- **Screening and systems:** criminal and maltreatment-registry checks, scheduling and documentation software, and EVV for Medicaid.
- **Working capital:** enough cash to cover several months of wages, insurance, and overhead before Medicaid payments are steady — the cost new owners most often underestimate.

In-home care is not equipment-heavy, which keeps startup lighter than facility-based care, but the working-capital runway to licensure and your first Medicaid payments is real — fund it deliberately.

Two Worked Examples

Example 1 — A private-pay start in Little Rock

You want to open quickly. You form an Arkansas LLC, name a qualified administrator, secure a registered nurse, bind insurance and workers' comp, and adopt your policies. You apply to ADH for your Private Care Agency license and prepare for the survey. You hire two aides, complete their criminal and maltreatment-registry checks, and put them through the forty-hour training. Your first client comes from a hospital discharge planner whose patient needs help bathing, dressing, and preparing meals at home after a stay — private pay, billed weekly. As families ask about Medicaid, you enroll as an Arkansas Medicaid provider so you can serve Personal Care and ARChoices clients too. Referral targets you would build in Little Rock include the major hospitals, skilled nursing and rehab facilities, hospices, and senior communities across the metro.

The lesson of this example is that starting private-pay is a legitimate on-ramp, not a lesser choice. It lets you build an aide team, referral relationships, and operating habits while your license and Medicaid enrollment come through — so that by the time you can bill Medicaid, you already have a working agency ready to add those clients rather than starting from zero.

Example 2 — Going after Medicaid in Northwest Arkansas or Fort Smith

You are aiming at the Medicaid population from the start. You secure your RN, form the business, and adopt policies built to the Rules for Private Care Agencies. You get your ADH Private Care Agency license, then enroll as an Arkansas Medicaid provider and get each caregiver a PIN and EVV. A DHS county office or an ARChoices care coordinator authorizes personal care for a client; for an ARChoices client the DHS registered nurse develops the person-centered service plan, and your aide delivers exactly the services on it. You capture EVV on every visit and coordinate changes back to the DHS RN. As you take Medicaid clients, you keep building the private-pay side through the same referral network — hospitals, skilled nursing, hospices, and senior communities — across Fayetteville, Springdale, Rogers, Bentonville, and Fort Smith.

The lesson of this second example is that the Medicaid path front-loads the work — the license, the RN, the aide training, the enrollment, and EVV — but opens the door to the population that makes up the bulk of long-term in-home care in Arkansas. Owners who go straight for Medicaid are trading a slower start for a larger, more durable client base, anchored by relationships with the DHS county offices, the ARChoices program, and the PASSEs that authorize the care.

Who to Call & What to Ask

For...	Who to contact
Forming your business	Arkansas Secretary of State (sos.arkansas.gov). Ask: what do I file to register my LLC, and what are the ongoing requirements?
Licensing your agency	Arkansas Department of Health (ADH), Health Facility Services — Private Care Agency license (healthy.arkansas.gov). Ask: what does a complete Private Care Agency application require, whether a Permit of Approval applies to my model, and what are the current fee and survey timeframes?
Background & registry checks	Arkansas State Police (criminal background checks) and the DHS Adult and Child Maltreatment Central Registries. Ask: how do I complete the criminal and maltreatment-registry checks on my staff?
Enrolling as a Medicaid provider	Arkansas Medicaid, through the Provider Portal (fiscal agent Gainwell Technologies, 1-800-457-4454). Ask: how do I enroll as a personal care provider, get caregiver PINs, and set up EVV?
ARChoices, PASSE & IndependentChoices	Arkansas DHS, Aging, Adult, & Behavioral Health Services

For...	Who to contact
	(humanservices.arkansas.gov), your regional PASSE, and the IndependentChoices fiscal agent (Palco). Ask: how do I serve members and coordinate service plans?
The aging & disability network	Your Area Agency on Aging and the Eldercare Locator (1-800-677-1116). Ask: how do families and Medicaid clients find and choose in-home care?
Wage & hour rules	Arkansas Department of Labor and Licensing and the U.S. Department of Labor. Ask: what are the current Arkansas minimum wage and the FLSA home care overtime rules?
Reporting maltreatment	Adult Maltreatment Hotline (Adult Protective Services) 1-800-482-8049; Child Abuse Hotline 1-800-482-5964; 911 for emergencies.

Key Terms

- **Private Care Agency license:** the ADH license a non-medical personal care agency needs in Arkansas, under Ark. Code Ann. § 20-10-2301 et seq.
- **ADH, Health Facility Services:** the Arkansas Department of Health division that licenses and surveys private care agencies.
- **Act 853 of 2025:** the law that moved licensing to ADH, ended the old DHS/DPSQA certification, made Medicaid a separate enrollment, and allowed a single primary office.
- **Personal Care (State Plan):** Arkansas Medicaid's hands-on personal care benefit, with a monthly hour limit for adults absent a benefit extension.
- **ARChoices in Homecare:** Arkansas's home and community-based waiver; the DHS registered nurse develops the person-centered service plan.
- **PASSE:** Provider-led Arkansas Shared Savings Entity — managed care for members with behavioral health or developmental-disability needs.
- **IndependentChoices:** Arkansas's self-directed option, where the participant directs their own care.
- **The 40-hour training:** the personal care aide requirement — 24 classroom hours plus 16 practical hours under a registered nurse.
- **Maltreatment Central Registry:** the adult and child registries Arkansas checks for direct-care staff.
- **EVV:** Electronic Visit Verification, required for Medicaid in-home visits; caregivers enroll for a PIN.

Core Rules & References

- Ark. Code Ann. § 20-10-2301 et seq. — Private Care Agencies (the Arkansas Rules for Private Care Agencies; ADH, Health Facility Services)
- Act 853 of 2025 (HB1439) — moved private care agency licensing to ADH; ended DHS/DPSQA certification; separate Medicaid enrollment; single primary office
- Ark. Code Ann. § 20-10-801 et seq. — Home Health Agencies (Class A, B, C — skilled services; a separate license, not required for personal care)
- Ark. Code Ann. §§ 20-33-213 and 20-38-101 et seq. — criminal background checks for direct-care staff; adult and child maltreatment central registry checks
- Arkansas Adult and Long-Term Care Facility Resident Maltreatment Act — adult maltreatment reporting (Adult Maltreatment Hotline 1-800-482-8049)
- Arkansas Medicaid — Personal Care (State Plan), ARChoices in Homecare waiver, PASSE, and IndependentChoices; Electronic Visit Verification under the 21st Century Cures Act

This guide is a general educational resource for prospective Arkansas in-home care agency owners. It is not legal, tax, or licensing advice, and requirements, fees, and contacts change — and Act 853 of 2025 recently reshaped the rules. Verify every step, figure, and phone number with the agency named — the Arkansas Secretary of State, the Arkansas Department of Health (Health Facility Services), the Arkansas Department of Human Services, and the relevant payer — before you rely on it.

Provisions cited reflect Arkansas law at the time of writing.